

Gestational diabetes

» What is gestational diabetes? Does it mean I have diabetes, or my baby does?

There are two main types of diabetes, Type I and Type II (see p.141). However, if you do not usually suffer from either of these types of diabetes but you develop insulin resistance during your pregnancy, you have gestational diabetes mellitus—GDM. It's a condition that affects the mother, and potentially the baby's growth. GDM usually develops during the third trimester of pregnancy, after 24 weeks pregnant, and it will usually disappear once the baby has been born. However, if you have had gestational diabetes, you are more likely to go on to develop Type II diabetes in later life. Gestational diabetes occurs in between 2 and 5 percent of pregnancies worldwide.

» My mother has Type I diabetes, but I don't. Will I be screened for gestational diabetes?

Yes, you will be screened, but not specifically because of your family history. The US Preventive Services Task Force recommends that all pregnant women get screened for gestational diabetes after 24 weeks of pregnancy. Most doctors screen their patients between 24 and 28 weeks. The blood-glucose tolerance test can be done right in the doctor's office. Women of southern Asian, Afro-Caribbean, or African descent, women who have had a previous baby who weighed more than 9 lb (4.5 kg), who have had gestational diabetes in an earlier pregnancy, or who are obese are at greater risk for the condition.

» Will I have to have a C-section?

Not necessarily. Clinical guidelines state that gestational diabetes alone is not a good enough reason to recommend a C-section. However, one effect on the baby is to make her "large for dates." If this applies to your pregnancy, your obstetrician may recommend a C-section since your baby's size could make labor difficult or distressing for one or both of you (see below). You'll have ultrasounds throughout the final stages of pregnancy if you have gestational diabetes, so your baby's growth will be monitored carefully and you'll be able to make a fully informed decision about how you give birth, and to weigh your wishes against the advice of the doctors.

» Will I be allowed a home birth if I have gestational diabetes? What about an active birth?

It's unlikely. If your diabetes is severe, you will most likely need to be delivered on a labor ward because of the increased risks to you and your baby during labor and birth. You will also be attached to an IV of insulin and glucose, which will make moving around tricky.

Your doctor will meet with you at around 36 weeks pregnant to discuss the safest options for your baby's birth.

» How will gestational diabetes affect my baby? What about after birth?

GDM can cause a baby to become large for dates, but that is the only direct effect the condition has on the baby while she is in the uterus. This does increase the risk of labor complications, since the baby may be too large to pass through the pelvis, and has an increased risk of shoulder dystocia, when the head is delivered, but there is difficulty delivering the shoulders, so a planned C-section may be advised. There are other effects that can influence the baby's well-being. For example, gestational diabetes can cause too much amniotic fluid to be produced, which in turn can trigger premature labor, and there may be issues stabilizing the baby's blood sugar after birth. The condition also puts you at greater risk of preeclampsia, which can be dangerous for both you and your baby. Finally, studies show that babies born to mothers who had gestational diabetes are more prone to obesity in later life, with the domino effect of increasing their risk of developing Type II diabetes.



Blood glucose meter You'll be given a blood glucose meter. This allows you to check your glucose levels through a small pinprick sample of blood.

If you had gestational diabetes in a previous pregnancy you have a **67 percent** chance of developing it subsequent pregnancies.

Diabetic treatments



Food choices You will be advised to eat food that will help to keep your blood glucose as stable as possible. Whole-wheat bread and brown rice are both good for releasing energy gradually.

As a first step in your treatment, you'll be referred to a dietician, who will talk to you about how you can try to control your glucose levels through diet. Injecting yourself with insulin is something of a last resort.

» Food and drink

You will be given guidance on dietary changes and told specifically which foods and beverages you should consume.

» Exercise

Your doctor will talk to you about forms of exercise that are safe for you.

» Insulin

However, if you can't control your glucose using diet and exercise alone, you will need to lower the amount of glucose in your blood either by taking metformin orally, or through insulin injections. If you need injections, a specialist will talk you through this.